Hip Labral Repair Protocol Phase 1

Purpose

Maximum Protection

Typical Time Frame

0-2 Weeks

Precautions

- Avoid hip flexion > 90 degrees for 2 weeks
- Avoid hip extension > 0 degrees
- Avoid ER > 30 degrees (first 2 weeks)
- TTWB (20%) or WB 50% first 2 weeks
- Avoid sitting for >1-2 hours (avoid knees above hips)
- No active straight leg raises (flexion and extension)

Criteria to Pass

- Patient's average VAS score < 4/10
- Ambulation with no antalgic gait
- Normalize gait pattern with crutches

Priority List & Suggested Exercises

1. **Hip Range of Motion/Stretching**
   a. PROM: Circumduction (small CW & CCW) at 30 flexion & 70 degrees of hip flexion, supine IR & ER rolls (no >30 degrees), supine abduction
   b. Manual Techniques: contract relax for IR/ER
   c. Extension: Hip flexor stretch with knee to chest with non surgical leg > Prone lying > prone on elbows > prone knee bends (active)
   d. Flexion: Heel slides > Ball assisted hip flexion
   e. ER/IR: rotations on stool
   f. Bike (with seat raised and no resistance) *** No recumbent bike
2. **Core/Hip strengthening**
   a. Isometrics: TA setting, hooklying ABD/ADD holds, gluteal sets, quad sets > SAQ > LAQ, hooklying hamstrings
3. **Gait Training (Mindful of WB precautions)**
   a. Weight shifting with UE assistance
   b. Gait training with crutches

Special Considerations

- Avoid pain and pinching in the hip with stretching and exercises at all times (RPE 5/10; VAS 2/10)
- Avoid long axis hip distraction of the hip for first 8 weeks for labral repair and 12 weeks for labral recontstruction
- Manage swelling and pain with icing 2-3x/day (directly after PT session or HEP)
- Avoid standing, walking, and sitting for prolonged periods
- Incision: inspect frequently to ensure healing and closing
- Programming:
  - Emphasize gentle ROM and core stabilization retraining

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Hip Labral Repair Protocol Phase 2

Purpose

Protection

Typical Time Frame

2-8 Weeks

Precautions

- Avoid aggressive stretching (especially hip ext/ER)
- Avoid pain and pinching in the hip with stretching and exercises at all times (RPE 5/10; VAS 2/10)
- No grade 3-4 joint mobilizations for 6 weeks
- Avoid long axis hip distraction of the hip for first 8 weeks for labral repair and 12 weeks for labral reconstruction

Criteria to Pass

- 90% Hip ROM compared to contralateral side
- SLS x 30sec without LOB with good hip control
- 20 consecutive 4-way SLR with good ROM and hip control
- Core Series:
  - SL Bridge Hold x10sec
  - Modified Side Plank Hold x10sec
  - Deadbug Hold x10sec

Priority List & Suggested Exercises

1. **Hip Range of Motion/Stretching**
   a. Continue phase 1 stretches as needed
   b. PROM
   c. Manual Techniques: Joint mobs (grade 1 & 2); contract relax for IR/ER
   d. Static stretching: prone quad stretch, ½ kneeling hip flexor (pelvic tilt **without** forward lunge), modified butterfly stretch
   e. Quadruped rock backs
   f. Standing hip rotations on stool (IR/ER) & standing extension on stool
   g. Standing knee flexion
   h. Bike (no resistance until 6 weeks)
   i. Gentle hip circles
2. **Hip Strengthening**
   a. 3-way SLR with (ABD, ADD, Ext) (isometric holds 5-10 sec before advancing to reps: keep sets low 1-2)
   b. Hip flexion: AAROM with physioball > active knee flexion to chest > SLR
   c. Double leg bridge > Standing hip extension > Quadruped hip extension
   d. Clamshell holds > Reps > Resisted
   e. Prone Resisted IR/ER > Seated IR/ER
   f. Shuttle press variations (DL, SL, sidelying)
3. **Core strengthening**
   a. Double leg bridge hold > SL bridge hold
   b. Modified side plank (knees down) > Modified side plank w/ abduction
   c. Deadbug progression
   d. Birddog progression
   e. Lumbopelvic Stabilization Focus: Hooklying lumbar rotation > Prone hip IR/ER
   f. Pallof press
   g. Cable column rotation
4. **Gait & Balance Training**
   - Tandem Stance > SLS no UE support > SL balance reaches
   - Gait training on treadmill (holding on with UE)
   - Side stepping on treadmill
   - Low hurdle (reciprocal pattern) > high hurdle (step to)
   - Calf Raises

Special Considerations

- Manage swelling and pain with icing 1-2x/day (directly after PT session or HEP)
- Avoid standing, walking, and sitting for prolonged periods
- Incision: inspect frequently to ensure healing and closing
- Programming:
  - Emphasize core stabilization with static contractions and building endurance

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Hip Labral Repair Protocol Phase 3

Purpose

Functional Restoration

Typical Time Frame

6-12 Weeks

Precautions

- Avoid aggressive stretching (especially hip ext/ER)
- Avoid pain and pinching in the hip with stretching and exercises at all times (RPE 5/10; VAS 2/10)
- Avoid long axis hip distraction of the hip for first 8 weeks for labral repair and 12 weeks for labral reconstruction

Criteria to Pass

- Walk 1 mile w/o compensation or AD
- Ascending/Descending stairs in reciprocal pattern
- 5 steps ups (10" step)
- 5 lateral step downs (8" step)
- Squat symmetrically to 90°
- Y balance reach 95% of uninvolved side without hip strategy
- Hip outcome score: ADL category 90% or greater score

Priority List & Suggested Exercises

1. **Hip Range of Motion**
   a. Continue Phase 2 exercises
   b. TRX squat
   c. TRX Cossack Squat
2. **Gait & Balance Training**
   a. 3 way balance reaches > star drill
   b. Gait series: ½ kneeling hip flexor stretch > PPU > Prone hip extension > retro walking > Treadmill walking
   c. Low hurdle (reciprocal pattern) > high hurdle (step to) > high hurdle (reciprocal pattern)
   d. Calf Raises
3. **Functional Strength**
   a. Leg extensions
   b. Leg curls (seated or prone)
   c. Step ups progression
   d. LSD progression
   e. Double leg shuttle press > Wall sits > wall squats > Squat to box > air squats
4. **Hip strengthening**
   a. Cable resisted 4 way hip strengthening
   b. Bridge progression
   c. Seated hip flexion > resisted > Standing hip flexion > resisted
   d. Quadruped hip extension on bench
   e. Resisted Clamshells, Reverse clamshells
   f. Stork stance holds > reps
   g. Band resisted Seated IR/ER > quadruped
   h. Hip hike holds > reps
5. **Core Strengthening**
   a. Continue Phase 2 core strengthening
   b. Front Planks
   c. Chop & Lifts
   d. Rolling patterns
   e. Hyperextension holds > hyperextension reps

Special Considerations

- Joint mobilizations can be added (grade 3-4 beginning at 6 weeks) only if neccessary
- Incision: Should be closed by 6 weeks & can begin aquatic therapy as an option
- STM to adductors, illipsoas, glutes, & hip flexors to address myofascial pain or restrictions
- Manage swelling and pain with icing (directly after PT session, HEP, or prolonged time on your feet)
- Cardio options: walking (gradaully increase time/distance); ellpitical with low resistance
- Consider tendinopathy diagnoses and focus treatment as necessary (many times their is a known diagnosis pre-surgery)

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Hip Labral Repair Protocol Phase 4

Purpose

Progressive Overload

Typical Time Frame

10-18 Weeks

Precautions

- Avoid exercises that are painful and cause swelling
- Don’t initiate running

Criteria to Pass

- 30 lateral step downs (8" step, back flat against wall) with no hip compensation
- 15 forward step downs (8" step)
- 25 SL bridges on bench (knee bent at 20°)
- 25 SL calf raises (10° incline to 60bpm metronome)
- 25 SL squats to box (20") with minimal trunk flexion

Priority List & Suggested Exercises

1. **Functional Strength**
   a. Squat progression > SL squat progression
   b. Step ups progression
   c. LSD progression
   d. Forward step down progression
   e. Leg extensions
   f. Leg curls (seated or prone)
   g. Lunge Progression
   h. RDL Progression
   i. Calf Raise Variations
2. **Hip Strengthening**
   a. Cable resisted 4 way hip strengthening
   b. Hip Thrust Progression
   c. Standing hip flexion > resisted > wall hip flexion
   d. Quadruped hip extension on bench w/ ankle weight
   e. Resisted Clamshells, Reverse clamshells
   f. Stork stance holds > reps
   g. Quadruped ER/IR > Resisted
   h. Copenhagen Progression
3. **Core Strengthening/Balance**
   a. Continue Phase 3 core strengthening
   b. SL RDL reaches
   c. 4 corner balance reach
   d. KB pass through
   e. ½ kneeling halos > Tall kneeling halos
4. **Range of Motion**
   a. Focus on problem areas
   b. Couch stretch
   c. Hip 90/90
   d. Runner’s stretch
   e. Sumo squat
   f. World’s greatest stretch

Special Considerations

- Ensure that ROM is = to uninvolved LE during this phase
- Motor Control: If there is still a difference between AROM/PROM focus on end range strengthening and isometrics
- Walking program as part of HEP building up to 30minutes at 3.5mph

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Hip Labral Repair Protocol Phase 5

Purpose

Intro to Impact

Typical Time Frame

16-20 Weeks

Precautions

- Don’t initiate running until patient has passed the criteria below for return to jogging

Criteria to Pass

To Begin Jogging: (Use Strengthlevel.com)

- 12 reps at 70% of Novice Goblet Squats
- 12 reps at 70% of Novice Forward Lunges
- 12 reps at 70% of Novice DB RDLs
- 15 SL hops in place w/ good load acceptance & landing in same spot

To Move Onto Phase 6:

- ATHLETES: Run ½ mile w/o limping
- GENERAL POP: Jog 5min w/o limping
- Sprint w/o compensation
- Land symmetrical <10% difference w/ 5 squat jumps on Force Plates
- Decel efficiently in the sagittal plane

Priority List & Suggested Exercises

1. **Anterior Chain Strength**
   a. Double leg extension machine > Single leg
   b. Leg press machine
2. **Functional Strength**
   a. Lunge progression
   b. Loaded step up variations
   c. Loaded goblet squat progression
   d. Loaded LSD & Forward step downs
3. **Posterior Chain Strength**
   a. Hamstring curl progression
   b. Hip thruster progression
   c. Deadlift/RDL variations
   d. Nordic progression
   e. Calf raise variations
4. **Agility/Impact**
   a. Deceleration: snap down progression > low impac depth drops off step
   b. Mini jumps > Jump progression
   c. Jogging progression > hop & stop > UE assisted treadmill intervals
   d. Multiplanar hops > ladder decels > change of pace > stop & goes
5. **Lateral Chain Strength**
   a. Lateral squat variations
   b. Copenhagens
   c. Anti-rotation and balance training

Special Considerations

- Stretching: Static and dynamic stretching should continue
- Compound movements (loaded) should be progressed to increase hypertrophy and strength
- Programming:
  - 2:1:1 anterior chain to posterior chain to impact exercises
  - Increase sets to 4 with reps 6-12 specifically for anterior and functional exercises
  - No > 2-3 sets of 3-5 reps for agility/impact exercises
  - Train both sides evenly
  - Add in hip strengthening for accessory work if they continue to have any challenges
- Begin run/walk program once cleared for jogging

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Hip Labral Repair Protocol Phase 6

Purpose

Deceleration

Typical Time Frame

4-6 Months

Precautions

- Don’t initiate cutting until patient has passed the criteria below for return to cutting

Criteria to Pass

To Begin Cutting: (Use Strengthlevel.com)

- 8 reps at 80% of Intermediate Goblet Squats
- 8 reps at 80% of Intermediate Forward Lunges
- 8 reps at 80% of Intermediate DB RDLs

To Move Onto Phase 7:

- SL hop test >80% LSI with average of 3 trials
- <20% deficit for Deceleration RFD and Peak Force with landing w/ 3 SL Countermovement Jump on Force Plates

Priority List & Suggested Exercises

1. **Agility/Impact**
   a. Medicine ball drops
   b. Weighted hops > 3-way hurdle hop
   c. Hop over hurdle in sagittal plane > hurdle hop into lateral plane
   d. Hop over hurdle (lateral plane) > transition hops
   e. Lateral shuffles, reactive cutting, pro agility drill, T-test drill
   f. Cutting footwork drill > cone cutting > reactive cone cutting
2. **Anterior Chain Strength**
   a. Heavy SL machine extension
   b. Advanced Shuttle Press variations
   c. Leg Extensions on Exxentric
   d. Squat Variations
3. **Functional Strength**
   a. Continue with Phase 5 exercises
   b. Add more load to functional movements
   c. Exxentric: squat, split squat
4. **Posterior Chain Strength**
   a. Hamstring curl progression
   b. Hip thruster progression
   c. Deadlift/RDL variations
   d. Nordic progression
   e. KB swings
   f. Calf raise variations
5. **Lateral Chain Strength**
   a. Lateral step ups
   b. Lateral squat variations
   c. Exxentric: lateral lunges
   d. Rotational deadlifts

Special Considerations

- Icing & Stretching: At the end of session or at home; Continuing stretching and recovery strategies)
- Programming:
  - No > 3-4 sets of 3-5 reps agility
  - 2:1 Power to Strength ratio
  - Undulated Loading Model: Example: Mon - Mod intensity; Wed - High intensity; Fri - High volume
  - Rest Days: At least 3 days off
  - Tempo: 0-0-0 Decelerate the load and transition to acceleration as fast as possible
  - Plyometrics should be done prior to strengthening in order to avoid fatigue
  - Once patient can tolerate advanced plyometrics for multiple sets/reps without pain and with good form you can begin supersetting strength with power
- Conditioning: Begin through cross training on elliptical, bike, rower, jogging

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Hip Labral Repair Protocol Phase 7

Purpose	Precautions
Return to Sport	
Typical Time Frame	
6-8 Months	

Criteria to Pass

- SL hop test >95% LSI with average of 3 trials
- Triple hop test >95% LSI with average of 3 trials
- Hip outcome score: Sport category 90% or greater score

Priority List & Suggested Exercises

1. **Agility/Impact**
   a. Continue with phase 6 agility/impact exercises until performing proficiently
   b. Depth jump progression
2. **Complex Training:** *Compound Exercise at 80% 1RM (4 reps) followed by a plyometric jump exercise that mimic the same motor pattern (3 reps); 3-6 sets*
   a. **Anterior Chain Complex Example:** Front Squats followed by Squat Jumps
   b. **Posterior Chain Complex Example:** Barbell RDLs followedy by KB Swings
   c. **Lateral Chain Complex Example:** Lateral Lunges > Skaters
3. **Contrast Training**
   a. *Perform a maximal or near maximal lift (90-97% 1RM) followed by a drop set performed at 50-70% 1RM. Perform at maximal velocity.*
4. **French Contrast Training**
   a. *4 exercises performed in succession*
   b. Heavy compound exercise (80-90% 1RM) 1-4 reps
   c. Plyometric jump 3 reps
   d. Drop set or weighted jump (30% 1RM) 3 reps
   e. Accelerated jump (usually band assisted) 2-3 reps

Special Considerations

- Icing: At the end of session or at home
- Prioritize Recovery: 2-3 days off/wk
- Training Scheme: Master complex training > contrast training > french contrast training
- Patients should have about 1 month of sport specific movements before beginning contact. Build confidence with stopping, cutting, training, game like movements/plays, etc.
- Continue building strength and explosive movements
- Ramp up conditioning specific to sport

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